The American College of Obstetricians and Gynecologists
WOMEN'S HEALTH CARE PHYSICIANS

June 16, 2025

Robert F. Kennedy, Jr.
Secretary
Department of Health and Human Services
200 Independence Ave SW
Washington, DC 20201

Submitted electronically to: https://www.regulations.gov
**RE: CMS-0042-NC; Request for Information; Health Technology Ecosystem**

Dear Secretary Kennedy,

The American College of Obstetricians and Gynecologists (ACOG), representing more than 62,000 physicians and partners in women's health and the health of individuals seeking obstetric and gynecologic services, appreciates the Centers for Medicare & Medicaid Services (CMS) and the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology's (ASTP/ONC) efforts to seek input from the public on the health technology ecosystem that will improve access to and usability of digital health tools for Medicare beneficiaries. It is important to stress that ACOG members work with very sensitive patient information, and the effective adoption of digital tools that will lead to a secure, safe transmission of patient health data is of utmost importance.

As the national medical specialty society of women's health physicians, our comments will focus on the workflow of ob-gyns experience and the unique needs of those seeking obstetric and gynecologic services.

*Patient Needs*

ACOG firmly believes that policies should be designed to protect the privacy, security, and safety of all patients seeking care from ob-gyns. Strengthening regulations that prevent the unauthorized sharing of sensitive health data relevant to ob-gyns, including intimate partner violence (IPV) and elder abuse, when using digital products across healthcare settings is critical.

With about 55 percent of Medicare beneficiaries being women, ACOG strongly encourages CMS and ASTP/ONC to enact policies that protect patients from violence and abuse.[i] Sexual violence, physical violence, and/or stalking victimization by an intimate partner has been reported by 47.3 percent of women in the United States.[ii] The Pregnancy Risk Assessment Monitoring System (PRAMS) found 5.4 percent of women reported any type of IPV during pregnancy.[iii] Additionally, one in ten older adults experience elder abuse, and the majority reported involve women being abused.[iv] ACOG recommends that ob-gyns incorporate screening for IPV into the routine medical history regardless, if abuse is suspected and reminds ob-gyns that documentation in the medical record is essential for

any potential legal proceedings.[v] Similarly, ob-gyn practices must ensure that any communication with the patient is secure in order to avoid retaliation from the abuser.[vi] **Therefore, ACOG requests that CMS and ASTP/ONC ensure digital communications, such as a digital application, utilize multifactor authentication**.

*Digital Health Apps*

The digital health gap remains a critical barrier to equitable healthcare access, with 35 percent of patients living in rural areas without access to high-speed internet, and only 14 percent of rural hospitals with a telehealth program.[vii] The plan by CMS and ASTP/ONC to advance the use of digital health apps that will give patients the opportunity to view, manage, utilize and share their electronic health information (EHI) with providers is a good step in promoting patient access to their health care data and empowering them to make informed decisions about their care. As patient engagement is fundamental to the success of digital health innovation, however, the cost of acquiring this digital health ecosystem can be daunting for people in rural areas, limiting their affordability. As such, it is **imperative that CMS and its partners, including the Federal Communications Commission, provide funding opportunities in the form of grants or incentives that will offer financial support for high-speed internet and technical assistance to adopt these innovative technologies**.

Ob-gyns must maintain the confidentiality of documentation that, if shared without measure, could endanger women or make them more vulnerable to violence or discrimination. Work must be done to ensure EHI vendors can safeguard sensitive data, and consent management software is widely available. With this plan in mind, it is equally important to recognize that the privacy of sensitive health data is critical to maintaining trust with patients, so care should be taken by providers when sharing sensitive health data using digital health apps. As such, **we recommend a regulatory environment that promotes innovation while ensuring patient safety and data privacy to ensure sensitive health is not inappropriately shared.**

*Data Exchange*

It is well known that the lack of standardized data formats and interoperability between different health information systems or electronic health record (EHR) 93 percent to 99 percent of systems can lead to inefficiencies and errors in data exchange. Despite 93 percent to 99 percent of hospitals adopting certified health IT systems, physicians are spending two to five hours on their EHR for every one hour of patient-facing time, and 80 percent of physicians report fatigue after 22 minutes of time on EHR use.[?] Data fragmentation complicates the aggregation and analysis of data from multiple sources, such as laboratory tests, imaging, and referrals, therefore hindering physician workflows. As such, developing standardized data formats such as in the United States Core Data for Interoperability (USCDI) and interoperable health information exchange such as in the Fast Healthcare Interoperability Resource (FHIR) will ensure seamless integration of health IT systems, thus reducing physician administrative burden. A study on the implementation of FHIR throughout multiple departments at the University of Utah's medical center estimates that an annual investment of $500,000 to $1 million in FHIR adoption and health informatics staffing has resulted in improved patient care and the provider experience, increased grant funding and research, and

enhanced the health system's ability to respond to change, such as adopting payment models or changes of care during the pandemic.[viii]

The plan by ASTP/ONC to update and expand the health IT certification criteria to leverage advancements in FHIR-based solutions and standards for better public health interoperability will improve data exchange and enable patient and providers' electronic access to health information using digital health tools. To that end, access to healthcare data through digital channels must be limited in certain circumstances to protect the trust and security of the physician-patient relationship and the safety of the patient. As CMS and ASTP/ONC push forward regulations to advance data interoperability and health information exchange, ACOG recommends incentivizing the quick adoption of FHIR - based Application Programming Interface (API) across all health IT systems that will support the implementation of the newer version of the USCDI data elements. It is also essential that USCDI data elements are consistent with current evidence-based guidelines, standards of care, and physician workflow. **ACOG recommends CMS and ASTP/ONC support the adoption of digital health tools that enable seamless integration across EHRs, payers, and third-party applications and support standard FHIR -based API that are secure and safe when transmitting patient health data.**

There have been unintended limitations of the CMS Promoting Interoperability Programs, especially related to physicians, practices and hospitals' ability to mine usable data for quality improvement and practice change. The focus on compliance with meaningful use and other regulations has hindered the customization of reports at the practice level, either unavailable, a costly upgrade, or necessitating a health IT expert on staff.[ix] here is an urgent need for infrastructure support, especially for physician practices, to better meet the needs of supporting a modern architecture. Overall, ACOG continues to be concerned with the financial and technological capabilities of physicians and practices to meet current and future standards for interoperability and the costs that are passed down to practice by the EHR vendors. As such, we recommend CMS and ASTP/ONC conduct an analysis on the current capabilities of practices in meeting technological standards, potential needs in the future, and the role of EHR vendors in implementation support. Doing such an analysis will help better understand the landscape of current capabilities and avoid inclusion of future requirements that unfairly penalize practices to work beyond their technological capabilities. Further, ONC should consider methods on how to better incentivize or support the uptake of technology to meet new requirements and encourage vendors to offer workable, reasonable solutions.

*Information Blocking*

In past regulatory rulemaking, ACOG has expressed concerns over the implementation of information blocking requirements imposed by the CURES Act. These concerns focus on the reality of the feasibility of these requirements on practices and EHR vendors alike, especially on the concept of data segmentation. We strongly believe that protecting certain data specific to women's health is essential for information blocking requirements and so compliance can be realized. For example, individuals who have experienced intimate partner violence, sexual assault, and other sensitive experiences that disproportionately affect women need that information protected within their medical records, and they may choose to withhold symptoms or circumstances essential out of

fear. This fear also delays or deters individuals from seeking needed health care, resulting in worse health outcomes. It is crucial to protect the patient-provider relationships by fostering trust in the health care system and securing the protected health information of patients. Ob-gyns must maintain the confidentiality of documentation related to care for sexually transmitted infections, substance use disorder, or other conditions that, if shared without measure, could endanger women or make them more vulnerable to discrimination. Any movement towards the implementation of disincentives should not occur until EHRs or digital health tools can assure sensitive data can be protected, and consent management software is widely available.

The Protecting Care Access Exception that was finalized by ASTP/ONC was added to ensure that HIPAA covered entities or business associates are not required to disclose certain reproductive health care information. The exception would apply regardless of whether another exception could also apply to an actor's practice in relevant scenarios. ACOG greatly appreciates ASTP/ONC's consideration of the concerns we and many other stakeholders have regarding the consistency across HIPAA updates and protections and information blocking requirements. Incorporating such exceptions has facilitated the complementary nature of these regulations and put the focus on protecting critically sensitive health care data. As such, **we strongly recommend maintaining 45 CFR part 171, subparts B to protect women from violence or abuse.**

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Thank you for the opportunity to comment on the Request for Information; Health Technology Ecosystem. We commend the effort of CMS and ASTP/ONC to promote innovation in digital health tools that will advance interoperability, data exchange and promote patient access to their health information and look forward to working with you to implement policies to improve the interoperability and privacy of sensitive patient health data. Should you wish to discuss these recommendations further, please contact Kehinde Taiwo, Clinical Coding & HIT, Senior Manager, at ktaiwo@acog.org.

Sincerely,

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American College of Obstetricians and Gynecologists

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